

CARDS FOR PREGNANCY AND BIRTH



The Pregnancy and Birth Mandala

Companion book to the card deck

THROUGH THE LENS OF PRE- AND PERINATAL EDUCATION

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Key to the mandala

THE RINGS, FROM THE CENTER OUT

-  **The center.** The Seed of Life holds the place of your baby.
-  **The child.** Her growth, and what she may be sensing.
-  **The mother.** Your body, hormones, and feelings.
-  **The educator.** Possible imprints and what helps.
-  **Connecting and healing.** The outer petals: practices for you and your baby.

AROUND THE CIRCLE, CLOCKWISE FROM THE TOP



First trimester

Months 1 to 3



Second trimester

Months 4 to 6



Third trimester

Months 7 to 9



Birth

Labor, birth, first hour

AROUND THE EDGE



The outer edge. The forty weeks, from week 1 at the top.

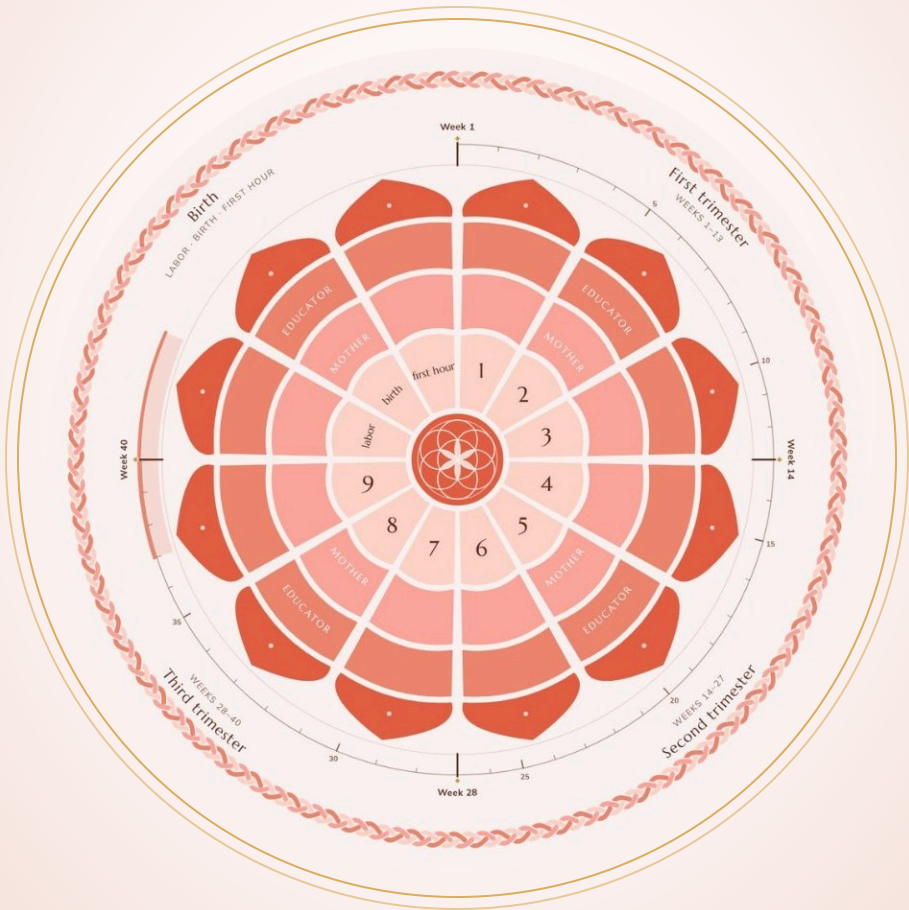


The braid. Three strands, for child, mother, and educator, holding the circle together.

See it come alive. The interactive mandala opens every month in detail, and with a due date entered it shows today, the birth window, and the moons of your pregnancy.

erinsarita.github.io/prenatal-mandala

THE PREGNANCY AND BIRTH MANDALA



A map for the journey, with a child at its heart

Pregnancy is often described in weeks, tests, and appointments. This deck invites you to see it another way: as a journey you and your baby take together, from the first days to the first hour after birth.

In pre- and perinatal education, the baby is understood as aware from the very beginning. She is taking in her world, and what she experiences in the womb, at birth, and in her first hours may leave imprints that shape her long after. This is not a weight to carry. It is an invitation to slow down, to notice, and to connect.

The cards come from a mandala, a circle organized around a center. At its center is your baby. Around her is you. Around you is what is good to know, and on the outer petals are practices for connecting with her and for your own healing.

The pages that follow lay the foundation: what this lens is and where it comes from, what an imprint is and how the body remembers, trauma and resilience, your nervous system, why speaking to your baby matters, and how each month is read. Then come the cards themselves, month by month, and people who can walk with you further.

There are no secrets you can keep from your baby,
so talk to her, and more importantly, listen.

Karen Strange, Simple Tools for Mothers

What is pre- and perinatal education?

Prenatal means before birth, and *perinatal* means around birth. Pre- and perinatal education draws on pre- and perinatal psychology, the study of how experiences from conception through pregnancy, birth, and the first year shape a person's body, relationships, and sense of self.

Its central idea is simple: your baby is a feeling, sensing participant in her own beginning, not a passenger. She responds to touch, sound, and your inner states long before birth, and she is learning about the world she is about to join.

WHERE IT COMES FROM

- 1924** Otto Rank's *The Trauma of Birth* proposes that being born leaves a lasting mark.
- 1930s** Konrad Lorenz describes *imprinting* in young geese, the word this field later borrows.
- 1960s–70s** Frank Lake and Stanislav Grof notice memories of womb and birth surfacing in adult therapy.
- 1981–83** Thomas Verny publishes *The Secret Life of the Unborn Child* and founds the association that becomes APPPAH.
- 1988** David Chamberlain's *Babies Remember Birth* gathers birth memories from children and adults.
- Since** Research on fetal programming, attachment, and the developing nervous system shows how early environments shape lifelong health.

Today APPPAH, the Association for Prenatal and Perinatal Psychology and Health, trains educators and practitioners to bring this understanding to families. This guide grew out of that training.

What is an imprint?

The word comes from Konrad Lorenz, who watched newly hatched goslings follow the first moving figure they saw, often Lorenz himself, and keep following. A first experience, met in a sensitive window, set a lasting pattern.

In pre- and perinatal education, an imprint is an impression left by early experience: conception, life in the womb, birth, and the first hours and days. Because it comes before words, it is held as body memory, in the nervous system and in patterns of feeling and response, rather than as a story she can tell.

Imprints shape her first answers to quiet questions: *Am I welcome? Is the world safe? When I reach out, will someone meet me?*

Three things to hold

Imprints can nourish. Welcome, calm, touch, and being spoken to leave impressions too. Most of this guide is about offering more of these.

An imprint is not a sentence. Later relationships, repair, and healing can reshape early patterns, at any age.

You carry imprints too. Pregnancy can stir your own earliest story. That is why every card holds a practice for your own healing.

Memory before words

Memory comes in two kinds. **Explicit memory** is what we can recall and put into words: facts, events, the story of a day. **Implicit memory** is held without any sense of remembering: in the body, in emotions, in what we expect and how we react.

Explicit memory depends on the hippocampus, which keeps maturing through the first years of life. This is why most of us recall little before age three or four. Implicit memory is at work much earlier, before birth.

WHAT RESEARCH SHOWS

Newborns prefer their mother's voice to other voices, a voice they came to know through the womb wall (DeCasper and Fifer, 1980).

Babies whose mothers read one story aloud in the last weeks of pregnancy preferred that story after birth (DeCasper and Spence, 1986).

Flavors from a mother's meals reach the amniotic fluid. Babies who tasted carrot this way later took to carrot-flavored cereal more readily (Mennella and colleagues, 2001).

She is learning before she can remember. Her early experiences are not lost: they are carried as implicit patterns of feeling and response (Siegel).

Why this guide says "may." The imprints on these pages are possible, not certain. Every baby meets her experiences with her own temperament, and the people around her shape what those experiences come to mean.

Memories that return

For a century, people in therapy have described experiences that seem to come from before words: a sense of the womb, of being born, of how they were received. In pre- and perinatal psychology these are called regression experiences, early impressions returning to conscious awareness.

WHAT HAS BEEN GATHERED

From the 1960s, psychiatrists Frank Lake and Stanislav Grof recorded clients reliving womb and birth experiences in deep therapeutic states.

Obstetrician David Cheek, working with hypnosis, described adults recalling the movements and circumstances of their own births.

David Chamberlain compared the birth memories of mothers and their children, recalled separately under hypnosis, and found many details matched (*Babies Remember Birth*).

Ham and Klimo found that adults' recalled sense of their mothers' feelings in pregnancy matched what their mothers reported.

Surveys in Japan found many young children speaking, unprompted, of the womb or their birth (Ikegawa).

A careful note: memory recalled in hypnosis or deep states can be shaped by suggestion and expectation, and mainstream science doubts that memories like these can form so early. What these accounts are is still being explored.

How early memory may be held

How could experience from before birth be carried? Part of the answer may be implicit memory: patterns of feeling held in the body and nervous system before there are words for them. Some practitioners speak of cellular or energetic memory, something carried that research has not yet found a way to measure.

However it is held, what people describe is often felt in the body first: in breath, in posture, in emotions that seem older than the story they know.

PREGNANCY AS A DOORWAY

Carrying a child often stirs a parent's own earliest story. Feelings may rise that seem bigger than the moment, or familiar in a way that is hard to name: a fear of being left, a longing to be held. In this lens, such moments may be your own early imprints surfacing, a natural part of becoming a parent.

Meeting them gently matters for you both. What is felt and understood can be soothed, and what is soothed is less likely to be passed on.

The healing practice on the back of each card is a place to begin. If what surfaces feels larger than you can hold, the people on page 38 work with exactly these early experiences.

Trauma and resilience

In this lens, trauma is defined less by the event than by what happens inside: an experience that is too much, too fast, or too soon, with too little support to meet it (Levine). The nervous system stays braced, as though the danger has not passed.

Not every hard experience becomes trauma. Researchers describe a range of stress: **positive** stress, brief and manageable; **tolerable** stress, buffered by caring relationships; and **toxic** stress, long, intense, and unbuffered (Center on the Developing Child, Harvard).

RESILIENCE

Resilience is the capacity to meet difficulty and find the way back to balance. It is not a fixed trait. It grows through relationship: being soothed, being understood, and knowing repair after rupture.

For your baby, resilience begins with co-regulation: her young nervous system borrows steadiness from yours. For you, it grows through support, rest, and tools you can reach for in the moment. Each return to calm is practice, for both of you, in the way back.

THE WINDOW OF TOLERANCE

Daniel Siegel describes a zone in which we can feel strong feelings and still think, connect, and respond. Above it we may feel flooded or panicked; below it, numb or far away. Support and resourcing widen this window over time, and labor asks for a wide one.

Each "Imprints and what helps" section holds both sides: what may be hard, and what builds resilience.

Your nervous system: an introduction to polyvagal theory

Polyvagal theory, introduced by Stephen Porges in 1994, describes how the autonomic nervous system responds to safety and danger. It offers a simple map of three states.

● Safe and connected

Breath is easy, face and voice soften, and you can rest, play, and bond. Oxytocin flows most freely here, in pregnancy and in labor.

● Mobilized

Heart and breath quicken, ready to fight or flee. Helpful in bursts, tiring when it lingers.

● Shut down

When danger feels overwhelming, the body may conserve: numbness, collapse, feeling far away.

Porges calls the body's constant, wordless scanning for safety *neuroception*. Your baby senses your state through your heartbeat, breath, voice, and chemistry, and her system begins to tune to yours.

Trauma can leave a nervous system stuck in mobilized or shut-down states. Resilience is the flexibility to move through them and find the way back to safety.

Polyvagal theory is widely used by therapists and educators, and some of its physiological details are still debated by researchers. Its map of states is offered here as a practical guide.

Resourcing yourself in the moment

A resource is anything that helps your nervous system feel a little steadier: a person, a place, a memory, a sensation in your body, something you can hold. Resourcing is turning toward these on purpose.

Knowing your nervous system widens your awareness. When you can notice "I am speeding up" or "I am drifting away," you have a choice, and the noticing itself is a step back toward presence, through pregnancy, labor, and the first hours.

TOOLS FOR THE MOMENT

Orient. Let your eyes move slowly around the room and rest on something pleasant.

Ground. Feel your feet on the floor, or your weight held by the chair.

Lengthen the out-breath. Breathe in, then let the breath out a little longer.

Touch. A hand on your heart or your belly, for you and for her.

Hum or sing. Gentle sound on a long exhale can help your system settle.

Reach out. A familiar voice or a hand to hold is among the strongest resources there are.

The back of each card holds practices with time set aside to connect: with your baby, with your partner, and with yourself. Return to them whenever you need resourcing.

Why it matters, for the benefit of all

Seeing pregnancy through this lens turns it from something that happens to you into a relationship you take part in. Knowing what may leave an impression lets you offer more of what nourishes, and meet hard moments with repair instead of guilt.

For your baby. A welcome felt from the start, a nervous system that learns calm from yours, and early experiences of being heard.

For you. Less fear and more confidence, a way to connect before you can hold her, and room for your own story to heal.

For your partner and family. A real part from the beginning: a voice she comes to know, a hand that answers her movements.

For those who care for you. A shared language for guarding the birth space and keeping mother and baby together.

For the generations to come. Patterns pass from parent to child. Each gentle beginning, and each repair, changes what is passed on.

Putting it into practice. Each card turns this understanding into small, daily acts: talking to her, answering her movements, settling yourself, guarding the birth space, and repairing after hard moments.

This is not about getting everything right. It is about noticing, connecting, and repairing, which every family can do.

Why speak to a baby who has no words?

Many practices on the cards ask you to talk to your baby: to tell her what is happening, what you feel, and what you hope. It can feel odd at first. She cannot understand words yet. So what reaches her?

WHAT REACHES HER

Your voice. From mid-pregnancy she hears, and your voice, carried through your own body, is the loudest and clearest in her world.

Rhythm and warmth. Long before meaning, she takes in the music of speech: its pace, its tone, its rise and fall.

Your body. When you speak tenderly, your breath slows, your heart steadies, and your muscles soften. She lives inside these changes.

Your chemistry. Warmth and connection bring oxytocin, the hormone of calm and closeness, and ease the stress hormones that matter most when they linger.

In pre- and perinatal education, words are understood as carriers of your attention, your intention, and your feeling. She may not know what "you are safe" means. She can feel what it is like when you mean it.

For partners. Your voice reaches her too, from just outside the womb. Babies come to know the voices they hear often, so a partner who speaks to her before birth is already familiar when she arrives.

She is listening

WHAT RESEARCH SHOWS

Before birth, babies' heart rates quicken to a recording of their mother's voice, and slow to a stranger's (Kisilevsky and colleagues, 2003).

Newborns prefer their mother's voice, and a story read aloud to them in the womb (DeCasper and Fifer, 1980; DeCasper and Spence, 1986).

Newborns' cries already carry the melody of the language spoken around them (Mampe and colleagues, 2009), and they recognize sounds heard often before birth (Partanen and colleagues, 2013).

Mothers who sang lullabies through pregnancy and after birth reported a stronger bond, and their newborns cried less (Persico and colleagues, 2017).

There is an intelligence in her that we cannot fully grasp, and research cannot yet say how much of your meaning she receives. What we can see is what it brings: she listens, comes to know your voice, and responds, and speaking to her changes you as well.

That is where its importance lies. Each time you speak to her, the relationship grows. She is seen, she is heard, she is accounted for, and the connection you build becomes a resource you can both return to.

HOW TO BEGIN

Start small: good morning, her name, a hand on your belly. Tell her what you are about to do. Name what you feel: "I am worried today, and it is not about you." Invite your partner to speak to her too. Then pause, and notice. She may answer with a move.

Each card offers words you can say to her. Take them as a starting place, and make them your own.

How each month is read

Every month in this guide follows the same pattern, moving from the center of the mandala outward.

● Your baby

Her growth, and what she may be sensing and taking in. Read it as a description of someone, not something.

● You

Your body, hormones, and feelings. Your inner states are her first environment, so caring for yourself is caring for her.

● Imprints and what helps

What this month's experiences may leave as an impression, and what tends to help: practices, people, and choices. These are possibilities to notice, not predictions.

● To explore further

The teachers and research behind the month, for when you want to read more deeply.

The back of each card carries the outer petals: practices for connecting with your baby, and one for your own healing.

Three questions to carry. What might she be experiencing? What am I experiencing? What would help us both?

One card at a time

- 1 Find your card.** Choose the card for the month you are in. The small wheel in its corner shows where it sits on the mandala.
- 2 Read the front.** It holds a short picture of your baby, of you, and of what is good to know this month.
- 3 Turn it over.** The back holds practices for connecting with your baby, with steps to follow, and one practice for your own healing.
- 4 Keep it close.** Set the card where you will see it: by your bed, on the fridge, in your bag. Return to its practices through the month.
- 5 Look ahead.** Read the next card a little before its month begins. Read the three birth cards early in the third trimester, so you have time to practice.

A few gentle notes

Share them. Many practices are for partners, too. Read the cards together.

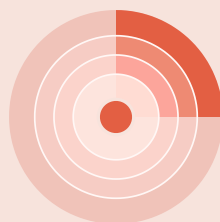
Go at your pace. Skip a practice that does not feel right. Return to it later, or not at all.

Write it down. A small journal beside the deck can hold what you notice and what surfaces.

Ask for help. If a practice stirs more than you can hold, page 38 lists people who can help.

The First Trimester

Beginnings



Life is often present before anyone knows it. These first weeks are full of rapid building, and of big feelings as the news arrives.

Your baby. From a cluster of cells to a moving fetus. Her heart begins to beat, every major organ begins, and the placenta takes root.

You. Hormones rise quickly, bringing fatigue, nausea, and tenderness. The news can bring joy, fear, and old feelings to the surface.

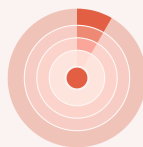
Through the lens. She is present before anyone knows she is there. How she is welcomed, in thought, word, and feeling, is among the first things she takes in.

The focus. Welcome her, find one way to settle yourself, and gather the people who will support you.

When to read. As early as you can, even before you feel ready.

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Month 1 · The Beginning

Your baby

Pregnancy is counted from the first day of your last period, so conception happens around week 2. About a week later, the tiny ball of cells settles into the lining of your womb, and the cells that will become the placenta begin reaching toward your blood supply. In the pre- and perinatal view, her experience begins here, often before anyone knows she is there.

You

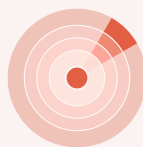
Hormones begin shifting at implantation. A missed period around week 4 is often the first sign, and some mothers notice fatigue, tender breasts, or light spotting. Life goes on as usual until the news arrives, and when it does, feelings can come all at once.

Imprints and what helps

The circumstances of conception, and whether she was hoped for, may become part of her earliest story. That story can be met without judgment. The most helpful foundation is simple: one way to settle yourself, such as grounding, and a circle of people you can turn to. If you have known loss or a long road to conceive, hope and fear can arrive together, and both are welcome.

TO EXPLORE FURTHER

David Chamberlain describes the prenaté as a conscious, sentient being, and gathered memories of the womb and birth shared by young children and adults. Ask yourself how you might welcome your baby as someone, not something, from the very start.



Month 2 · The Discovery

Your baby

Her heart begins to beat around week 6. The neural tube, the start of her brain and spinal cord, closes, and arm and leg buds appear. By week 8 every major organ system has begun, and the first touch receptors form around her mouth. She is already taking in how she is received.

You

Pregnancy hormones climb steeply, and nausea, deep fatigue, and a sharper sense of smell often peak now. Blood volume begins to rise. For many this is the month of discovery. Joy, shock, fear, and ambivalence can arrive together, and pregnancy may bring you closer to your own childhood, letting old feelings rise.

Imprints and what helps

How the news was first received may stay with her. In one study, mothers' first reactions matched feelings their grown children later recalled. Conflict with partners or parents can reach her too. What helps is a welcoming response from the people around you, and, if the news was hard, simple repair words spoken to her. Waiting for first scans is full of uncertainty. Hard feelings are natural and need no guilt.

TO EXPLORE FURTHER

Your baby senses your emotions through your body's chemistry, and later through your heartbeat, breath, and voice. Everyday stress is buffered by the placenta. It is long, unrelieved stress that matters most, and each return to calm helps (Ham and Klimo; Raffai).



Month 3 · Taking Root

Your baby

Now called a fetus, she moves, stretches, hiccups, and swallows amniotic fluid long before you can feel it. Sensitivity to touch spreads from her face across more of her body. Some of your chemistry, including stress hormones, reaches her through the placenta.

You

Nausea often eases near the end of this month as the placenta takes over hormone production, and your uterus begins to rise out of the pelvis. First prenatal visits and screenings usually happen now, and many families wait until now to share the news. Old feelings from childhood can surface in this window.

Imprints and what helps

Your own unresolved history, and the chemistry of your states, can reach her. What helps is a safe, trauma-informed space where you can share your story if you choose, and support when it feels big. How you nourish and rest yourself shapes her lifelong health, and small, steady steps count. Worry while waiting for screening results is natural, not a failure of trust.

TO EXPLORE FURTHER

Fetal programming, the idea that conditions in the womb shape lifelong health (Nathanielsz). The three models of care, technocratic, humanistic, and holistic (Davis-Floyd), and the Mother-Friendly Childbirth Initiative, to help you choose the care that fits you.

The Second Trimester



The Middle

Your baby begins to hear, you begin to feel her, and the relationship becomes something you can both sense.

Your baby. Her movements grow coordinated, hearing begins around weeks 18 to 20, and she begins to respond to voices, touch, and sound.

You. Energy often returns and your belly shows. Feeling her move can turn an idea into a relationship, and your thoughts begin to turn toward birth.

Through the lens. Her senses are opening, and relationship now runs both ways. When her movements and sounds are met, she learns she is heard.

The focus. Talk, sing, and answer her movements. Choose the stories you take in, and begin exploring your own birth story.

When to read. Around weeks 10 to 13, just before this trimester begins.

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Month 4 · The First Flutters

Your baby

Her movements grow smoother and more coordinated, and she makes facial expressions. Her inner ear and taste buds are taking shape. Her movements meet the walls of the womb, and soon they will meet your hand.

You

Energy often returns, and your belly begins to show. The placenta now makes most of the hormones of pregnancy. Some mothers, especially those who have been pregnant before, feel the first flutters. Relief and steadiness are common, and your sense of yourself begins to shift toward motherhood.

Imprints and what helps

Whether her movements are noticed and met may be one of her earliest lessons in relationship. Simple connection practices, for you and your partner, help, and a bonding program such as Pregnancy Dialogues can deepen them. As your pregnancy shows, others may share birth stories, many of them fearful. You can choose what you take in, set kind boundaries, and gather stories of births that went well.

TO EXPLORE FURTHER

Prenatal attachment, which grows as you turn your attention toward your baby. Karen Strange teaches that a baby's movements are her way of telling her story, and asks: can you slow down and be present to hear what she is saying?



Month 5 · The First Sounds

Your baby

Hearing begins. The first sounds reach her around weeks 18 to 20, and her hearing keeps sharpening through about week 25. Fine hair and a protective coating of vernix cover her skin, and sleep and wake cycles appear. Voices, music, laughter, and raised voices reach her as vibration through the womb wall.

You

Most mothers now feel movement. Relaxin softens the ligaments, and round ligament aches are common. The anatomy scan usually happens around week 20. Feeling her move can turn an idea into a relationship, and your partner can join in: a hand on the belly when she moves, a word or a song in answer.

Imprints and what helps

The sound world of your home may stay with her. In one study, grown children recalled music, laughter, and yelling their mothers also remembered. What helps is time to talk, sing, and play with her, and repair after loud or tense moments. If the anatomy scan brings unexpected news, you can ask what a result means, learn the options, and take time before deciding. Fear or grief is a natural response.

TO EXPLORE FURTHER

Your voice, carried through your own body, is the loudest and clearest sound she hears. Ham and Klimo describe a father whose "hoo, hoo" game from the fifth month led his newborn to stop nursing and turn toward his voice.



Month 6 · The Rhythm

Your baby

She begins to respond to sound, often moving or startling at a sudden loud noise. Her lungs begin making surfactant and her brain grows quickly. Her sleep is not yet divided into stages: she moves through loose cycles of rest and activity, and her brain waves are six to ten times slower than an adult's.

You

Your belly grows quickly. Back aches, heartburn, and swelling may begin, and a glucose screening is usually offered between weeks 24 and 28. Your thoughts may begin turning toward the birth, and your own birth story, and your mother's, may come to mind.

Imprints and what helps

The emotional climate over time matters most: ongoing tension or neglect, or warmth and contact. What helps is a steady rhythm of calm, a doula or childbirth class that fits you, and the rest you need, asked for in plain words. If the screening leads to a diagnosis such as gestational diabetes, it is information to guide your care, not a judgment on your body. Disappointment is natural, and it can sit beside caring well for yourself.

TO EXPLORE FURTHER

William Emerson's "elephant in the birthing room": everyone brings a birth story into the birth room. Oxytocin, the hormone of calm and connection, is easier to find in labor when it has been practiced in pregnancy (Strange).

The Third Trimester



The Ripening

Your baby grows, space grows tight, and both of your bodies prepare for birth.

Your baby. Her eyes open, dreaming sleep appears, and she comes to know your voice. Many babies settle head down, and her own lungs may help signal when labor begins.

You. Sleep grows harder and practice contractions stronger. Anticipation grows, and so can fear. Choices about position, monitoring, and induction may arise.

Through the lens. She is preparing for birth alongside you. How choices are made, and the feeling in the room, may become part of how she arrives.

The focus. Slow down, plan your birth space and support, practice staying in the decisions, and prepare for flexibility, not fear.

When to read. Around weeks 24 to 27, just before this trimester begins.

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Month 7 · Light and Voices

Your baby

Her eyes open and can sense light through your belly. She is gaining fat, practicing breathing, and moving between clear sleep states. REM sleep appears now, the active sleep linked with dreaming, and she spends much of each day in it. The voices she hears most will be familiar after birth.

You

The third trimester begins. Sleep grows harder, breath shorter, and practice contractions more noticeable, and prenatal visits come more often. Anticipation grows, and so can fear of birth.

Imprints and what helps

Fear in the room can become part of the birth: your own, your family's, or a provider's past cases. What helps is planning your birth space and choosing who will speak up for you; a doula can be a strong advocate at a hospital birth. It also helps to begin preparing for flexibility, not fear. Exploring other paths, such as a longer labor, an induction, or a cesarean, lets you meet a change as a choice rather than a collapse of the plan.

TO EXPLORE FURTHER

Sarah Buckley's four hormone systems of birth: oxytocin for contractions and love, endorphins for easing pain, adrenaline for the final pushes, and prolactin for mothering. Your rights in labor: companions, freedom to move, food and drink, and informed consent (MFCI).



Month 8 · The Turning

Your baby

She gains weight quickly and space grows tight, so her movements change from tumbling to stretching and pushing. Studies first show her responding to your voice differently from other voices, and after birth she prefers it. Antibodies cross the placenta to prepare her immune system, and many babies settle head down now.

You

Your uterus presses up under your ribs and builds oxytocin receptors for labor, and your breasts may begin leaking colostrum. Nesting energy often arrives. If she is breech, a real decision lies ahead, and you deserve full information and room to choose.

Imprints and what helps

As space grows tight, she may feel held or hemmed in, and how decisions about her position are made may shape her experience, and yours. What helps is gentle movement, telling her what is happening, and practicing how you will stay in the decisions: asking about benefits, risks, and alternatives, taking a moment, and saying yes or no. If she comes early, skin-to-skin care in the NICU keeps her close. A baby doula and help planned for the weeks after birth are worth arranging now.

TO EXPLORE FURTHER

Breech options: waiting, gentle positioning, an external version, or a vaginal breech birth with a skilled provider (Fischbein and Freeze). Continuous monitoring in low-risk labors is linked to about 20 percent more cesareans; you can ask about listening in at intervals.



Month 9 · Ripening

Your baby

Her lungs finish maturing, and full term arrives at 39 to 40 weeks. Research suggests her own maturing lungs release a signal that helps start labor, so she may have a part in deciding when birth begins.

You

She may drop lower, easing your breath but adding pressure. Practice contractions grow stronger and your cervix begins to soften. Waiting can be hard, especially as pressure to induce grows, and you may feel ready, impatient, or afraid, sometimes all in one day.

Imprints and what helps

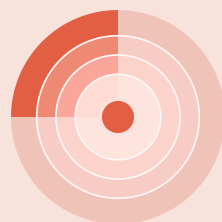
Being waited for, or hurried, may become part of her story. Labor usually begins when you are both ready, and her own body may help signal when. An induction is a choice to weigh, not an automatic step, and if one is chosen, telling her what is coming can soften it. What helps is an informed, unhurried decision, continuous support planned ahead, and a conversation about what you would want if plans change.

TO EXPLORE FURTHER

The ARRIVE trial and elective induction at 39 weeks. Synthetic oxytocin does not act like your own and is linked to more postpartum depression and anxiety (Kroll-Desrosiers). Expressing colostrum after 37 weeks, with your provider's approval, helped milk flow sooner after birth (Singh).

Birth

The Threshold



Hormones, the space, and the people present all shape how your baby arrives and how she is met.

Your baby. Labor holds her in rhythm and pressure. Birth brings light, air, and sound all at once. The first hour brings your skin, your smell, and your voice.

You. Oxytocin, endorphins, adrenaline, and prolactin carry you through, and they flow best when you feel safe, private, and unobserved.

Through the lens. Birth is her first great passage and her first welcome. Even when plans change, telling her what is happening and holding her close can soften what she meets.

The focus. Guard the space, tell her what is happening, and keep her close. If plans change, stay in the decisions and offer repair.

When to read. Early in the third trimester, around weeks 28 to 34, so you have time to practice and plan before the birth window opens at 37 weeks.

CARDS IN THIS SECTION

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Labor · The Threshold

Your baby

Contractions press and release her whole body like a long, firm embrace. Her own stress hormones surge, protecting her through each squeeze and preparing her lungs, and she receives some of your oxytocin and endorphins. Pressure, rhythm, and pause: the pace of labor becomes part of her story.

You

Oxytocin brings rhythmic contractions, and beta-endorphin eases pain and carries you inward. You labor best when you can move, eat and drink lightly, and follow your body, and when you feel private, safe, and unobserved, much like lovemaking.

Imprints and what helps

The pace and pressure of labor may stay with her: moments of feeling stuck, and a journey supported, or interrupted by synthetic oxytocin, forceps, or vacuum. What helps is a guarded space: dim light, quiet, and only people you know and trust, with no routine procedures without a reason. Pain is a normal part of labor, but suffering is something else; choosing relief is a sound choice that needs no guilt. If plans change, three things help you stay present: someone by your side, clear explanations, and time to respond.

TO EXPLORE FURTHER

Sarah Buckley's "undisturbed birth": when labor feels watched or unsafe, adrenaline rises and can slow oxytocin, and with it labor. Ina May Gaskin reminds us that labor is part of nature, and that walking and eating belong in it.



Birth · Through the Gate

Your baby

In the final push, a surge of noradrenaline makes her alert, often with wide open eyes at your first meeting. She takes her first breath, and her circulation changes course within minutes. Light, air, sound, and gravity arrive all at once.

You

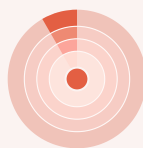
Near the end, a surge of adrenaline gives you strength for the final pushes, the fetal ejection reflex. Many mothers feel a burst of energy and alertness to meet the baby. Pain medication, such as an epidural or opioids, can soften this surge, so you may both be less alert when you first meet.

Imprints and what helps

Every physiological birth event may leave an imprint, and harsh lights, noise, routine suctioning, and tight wrapping can overwhelm a newborn. What helps is a calm arrival, delayed cord clamping, and a gentle cesarean if surgery is needed. If the birth takes an unexpected turn, she can be told what is happening, held close as soon as possible, and offered repair words afterward. A birth that changed course is not a failure.

TO EXPLORE FURTHER

A vaginal birth helps clear her lungs and seeds her with your beneficial microbes; skin to skin and breastfeeding help close the gap after a cesarean. Thomas Verny on birth and early imprints. Karen Strange on rupture and repair.



First Hour · The Welcome

Your baby

Placed skin to skin, she warms, steadies her heart and breathing, and may crawl toward the breast on her own. Skin, smell, heartbeat, and your voice, now from the outside. A newborn may not know she is out, or that she made it, until she is told and held.

You

Skin to skin, oxytocin and prolactin peak. Oxytocin helps the placenta release and protects against bleeding, and prolactin begins milk and mothering. Awe, relief, exhaustion, and the first look at your baby may arrive together.

Imprints and what helps

Welcome or separation may be among her first lessons about the world. What helps is keeping you together: skin to skin, breastfeeding, and rooming-in, with newborn checks on your chest and the first bath waiting. If the golden hour does not happen, through separation or medical care, the bond is not lost. Relationships are built over time, and repair can begin as soon as you are together. Mixed feelings in the days after are natural and need no guilt.

TO EXPLORE FURTHER

Humanistic care keeps mother and baby together; taking the baby to a nursery for bathing is not part of this model (Davis-Floyd). Watch in the weeks after for lingering distress, such as flashbacks, numbness, or avoidance, and reach out for support (Seng and Taylor).

Seven threads behind every card

Most practices on the cards grow from Karen Strange's *Simple Tools for Mothers*. Knowing the threads behind them helps you make the practices your own.

- 1 You are her regulator.** She forms around what you feel and experience. When you settle, she learns how settling feels.
- 2 Slow down.** Grounding brings you into your body; pacing matches her slower rhythm. Her brain waves are six to ten times slower than yours.
- 3 Talk to her, and listen.** Tell her what you will do before you do it, tell her what is going on, tell her what you would like, and tell her the story of what happened.
- 4 Your feelings are yours.** She feels what you feel. Telling her "this is about me, not you" begins healthy separateness.
- 5 Oxytocin moments.** Pleasure on purpose wires her system to move from stress back to calm, and gives you a groove to return to.
- 6 Rupture and repair.** Hard moments come into every relationship. Returning, naming what happened, and reconnecting builds deeper trust.
- 7 Someone for her, too.** A "baby doula" follows your baby's journey through birth, the way a doula supports you.

It is not just what you do or say that matters, but how you are on the inside.

Karen Strange

People who can walk with you

The healing practices on the cards are gentle starting points. When something stirs more than you can hold, or you want to go further, these people and approaches can help.

PRACTITIONERS

Pre- and perinatal practitioners and educators work with bonding, early imprints, and birth stories. APPPAH lists trained practitioners.

Perinatal mental health specialists support anxiety, depression, and trauma in pregnancy and after. Look for the PMH-C credential, through Postpartum Support International.

Trauma-informed psychotherapists help with history that surfaces, at a pace that feels safe.

Doulas for birth and the weeks after, and a baby doula to follow your baby's journey.

Childbirth educators, bonding programs such as Pregnancy Dialogues, and **lactation consultants** (IBCLC).

MODALITIES

Prenatal and birth therapy, in the tradition of Ray Castellino and William Emerson, works with the imprints of conception, womb life, and birth, for babies, children, and adults.

Somatic Experiencing gently releases stress and trauma held in the body.

Craniosacral therapy offers light, hands-on support for parents and newborns.

EMDR helps the mind process hard memories, including a difficult birth.

Birth story listening helps you tell your story, be heard, and find meaning in it.

An educator can help you find the right fit. Choose people who listen, who leave you feeling safe, and who let you set the pace. Keep their details on the next two pages.

My circles of support

Three rings of people around you and your baby.



Ring one · Care providers

Midwife, doctor, doula, lactation consultant, pediatrician

1	NAME	PROFESSION
	CONTACT	
2	NAME	PROFESSION
	CONTACT	
3	NAME	PROFESSION
	CONTACT	



Ring two · Family and friends

The people who will cook, listen, hold the baby, and hold you

4	NAME	RELATIONSHIP
	CONTACT	
5	NAME	RELATIONSHIP
	CONTACT	
6	NAME	RELATIONSHIP
	CONTACT	

MY CIRCLES OF SUPPORT · CONTINUED



Ring three · Pre- and perinatal support

Pre- and perinatal practitioners and educators, prenatal and birth therapy, Somatic Experiencing, craniosacral therapy, birth story listening, bonding programs

7	NAME	MODALITY
	CONTACT	

8	NAME	MODALITY
	CONTACT	

9	NAME	MODALITY
	CONTACT	

10	NAME	MODALITY
	CONTACT	

11	NAME	MODALITY
	CONTACT	

12	NAME	MODALITY
	CONTACT	

NOTES

Help, reading, and sources

IF YOU NEED SUPPORT NOW (UNITED STATES)

National Maternal Mental Health Hotline: call or text 1-833-TLC-MAMA (1-833-852-6262).

Postpartum Support International HelpLine: call or text 1-800-944-4773.

In a crisis, call or text 988. In an emergency, call 911.

FURTHER READING

Chamberlain, D. *Windows to the Womb; Babies Remember Birth.*

Verny, T., and Kelly, J. *The Secret Life of the Unborn Child.*

Buckley, S. *Gentle Birth, Gentle Mothering.*

Gaskin, I. M. *Ina May's Guide to Childbirth.*

Siegel, D., and Hartzell, M. *Parenting from the Inside Out.*

Strange, K. *Simple Tools for Mothers.*

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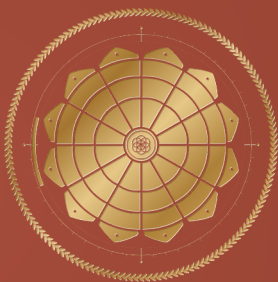
Seng, J., and Taylor, J. (2015). *Trauma Informed Care in the Perinatal Period*. Siegel, D. J. *The Developing Mind*.

Singh, G., et al. (2009). *MJAFI*, 65. Verny, T. R. Birth and the origins of violence.

NOTES

What I am noticing

Her movements, your feelings, what surfaced, what you want to remember.



At the center of all of this is a child
who is already present, already
listening, and already being shaped by
the world around her.

A card deck and guidebook for the journey of pregnancy and
birth, drawn from the Pregnancy and Birth Mandala and the
lens of pre- and perinatal education.

Created by Erin Singleton as part of APPPAH's Prenatal & Perinatal
Educator certification.

An educational companion, not medical advice. Talk with your care provider
about your own pregnancy.